

**العمر(تاريخ الميلاد) : 10-12-1966 المهنة : حاصل على دبلوم**

التوقيع: (-----)

التعليق على التشخيص : -----

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

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التشخيص النهائي :  
 خطة العلاج :  
 المستشفى :  
 قسم :  
 جهة العلاج :  
 نفقات :  
 ملاحظات :

رئيس  
المجلس الطبي المتخصص

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|----------------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> عواطف اسماعيل عبدالله اسماعيل<br><b>National ID:</b> 26612103200087 |  |
|----------------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

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| تقرير طبي |
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| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بأن نوضح فيما يلي تقريراً عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد أنه |
|----------------------------------------|

|                                                                                                              |
|--------------------------------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy |
|--------------------------------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



جمهورية مصر العربية  
بطاقة تحقيق الشخصية

عواطف

اسماعيل عبدالله اسماعيل

ب ١٢٩٠ دلكتا ٢

أول السلام - القاهرة

١٩٩٩/١٧/٢٠

٢ ٦٦ ١٢ ١٠ ٣٢ ٠٠٠ ٨٧

JL4702603

DMS  
عواطف



٢٠٢٢/٠٧

٢٦٦١٢١٠٣٢٠٠٠٨٧

حاصله على ديوان تجاره

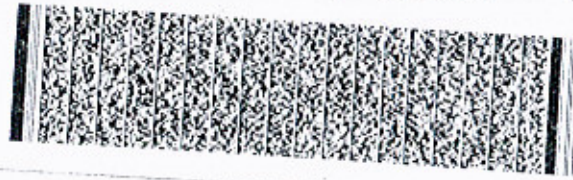
متزوجة

مسلمة

أنثى

محمد على محمد رضوان

البطاقة سارية حتى ٢٠٢٩/٠٧/٢٠





|                 |                                   |                    |                             |
|-----------------|-----------------------------------|--------------------|-----------------------------|
| Patient Name    | : الأستاذ / عواطف اسماعيل عبدالله | Request No.        | : 130&21                    |
| Patient No.     | : 26612103200087                  | Request Date       | : 17/10/2021                |
| Path No.        | : 2456/21                         | Verification Date  | : 03/11/2021                |
| Sex             | : Female                          | Financial Category | : اورام دار السلام - مبادره |
| Age             | : 53 Y                            | Department         | : Out Patient               |
| Referring Doc : |                                   |                    |                             |

## Pathology Report

### Clinical Data:

Left breast cancer, post chemotherapy.

### Nature of the specimen:

Right MRM.

### Gross Examination:

The specimens received, submitted as:

- 1- Labeled wide local excision; 9x5.5x5 cm, partially covered by skin ellipse; 8.5x4 cm. Sectioning showed an ill-defined grayish white mass; 2.5x2x1 cm.
- 2- Sentinel lymph node: 8 nodules. The largest was 2x1.5 cm.
- 3- Extra-medial margin: fibro-fatty tissue piece; 5x4 cm.
- 4- Axillary dissection; 5x3x2 cm, entangling 13 lymph nodes, Largest; 1.5x1 cm.

• Frozen section: Two fibrofatty tissue piece, medial margin and sentinel lymph nodes. The result was Positive micrometastasis in 2 out of 8 lymph nodes and medial margin with atypical hyperplasia and luminal microcalcification for extended margin.

### Microscopic examination:

- Histological type: Invasive duct carcinoma .
- Grade: II .
- Angioinvasion:
- Ductal carcinoma in situ: Focal insitu component; 10% .
- Tumor extent:
  - o Number: 1 mass .
  - o Size: 2x1.5 cm.
- Lymph node involvement: 2 out of 21 lymph nodes showed micrometastasis with extra-capsular extension. .
- Resection Margins: Extra margin showed positive lymphatic tumor emboli .
- Histologic response to therapy: poor response.



|                                                 |                                                |
|-------------------------------------------------|------------------------------------------------|
| Patient Name : الأستاذة / عواطف اسماعيل عبدالله | Request No. : 130821                           |
| Patient No. : 26612103200087                    | Request Date : 17/10/2021                      |
| Path No. : 2456/21                              | Verification Date : 03/11/2021                 |
| Sex : Female                                    | Financial Category : مبادره - اورام دار السلام |
| Age : 53 Y                                      | Department : Out Patient                       |
| Referring Doc :                                 |                                                |

### Pathology Report

Diagnosis:

Right MRM, Post neoadjuvant chemotherapy

- Invasive duct carcinoma. GII. with insitu component; 10%.
- Extra-medial margin showed positive lymphatic tumor emboli.
- Lymph node status: 2/21. with extracapsular extension.
- TNM: ypT2 ypN1.

?? how the WLE ??

Prof. Dr. Aml Hareedy.  
Dr. Mona Mohamad



**Women's Health Outreach Program**

Name: AWATEF ISMAIL ABD ALLAH

Date: 23.09.2021

**Contrast Enhanced Full field Digital Mammography**

**Clinical summary:** left breast malignant mass..

**Technique**

Pre & post contrast Mediolateral oblique and CC views

Targeted High resolution ultrasound

**Detailed Findings**

**Left breast:**

- mild back ground parenchymal enhancement.
- **Pre-contrast images:** ill defined asymmetrical area located at the upper outer quadrant.
- **Post-Contrast enhanced images demonstrated:** intense enhancing mass enhancement located at the UOQ corresponding to the aforementioned lesion located **4 cm from the nipple, measure 2.7 x2.4x2.5 cm .**
- No pathological axillary LN by ultrasound.

**Conclusion:**

-Left breast: (BIRADS 6= Pathology proved malignant ). unifocal UOQ pathologically enhancing mass.

Best regards,  
Dr Hanaa abo el magd



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|-----------------------------------------------|-------------------|
| Patient Name: Awaterf Ismail Abd Allah Ismail | Date : 14/09/2023 |
| Exam : ultrasound BREAST (BOTH SIDE )         | Age : 56 Year(s)  |
| Medical ID: 3530/2021                         | Pat. ID: 20340    |

**left BREAST MAMMOGRAPHY WITH BILATERAL US**

**Indication:**

Patient had right MRM in and left CBS

**Technique**

- Left-breast MLO and CC views
- Bilateral ultrasound

**Report:**

- The left breast is heterogeneously dense (ACR C)
- Left breast focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Clear left operative bed with no evidence of recurrence
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right MRM operative bed with no evidence of recurrent mass lesions
- Free bilateral infra and supra clavicular regions.

**Opinion:**

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Right MRM with free operative bed with no evidence of recurrent mass lesions .

Yours truly

Signature Dr. Engy Abdallah

14/09/2023 1:23 PM